



Hypertension

If Systolic BP ≥ 180 mmHG &/or Diastolic ≥ 110 mmHG

Confirm elevated BP reading in quiet room after 30 min ..
If STILL HIGH...

Examine fundi
(Papilledema)

DO ECG to R/O ischemic
change

Do URINALYSIS if available
(dipstick > proteins and
RBCs)

Hypertensive emergency

If symptomatic with signs of end-organ damage (HMOD):

- Confusion
- Headache or blurred vision
- Increasing chest pain or SOB
- Swelling or Edema

Hypertensive Urgency

If asymptomatic and no signs of end organ damage

Management :

- **Open IV Access**
- **Refer urgently to hospital**
- **Stabilize according to cause**

**If already on
hypertensive
medication:**

check
compliance
then restart or
optimize dose
of Rx ..

**If not on hypertensive
medication, consider:**

- **Directly** Start a **long acting antihypertensive** medication (according to guidelines) and follow up the patient. **(BETTER OPTION)**
- **May give Oral captopril 6.25 mg or 12.5 mg** avoid if patient is **volume overloaded** (no edema, pounding pulse, tachycardia, respiratory crackles)
- **Monitor the patient** for drop of (BP of 20 to 30 mmHg)

Urgent follow-up appointment for BP in few days